AS TO THE ADMISSIBILITY OF

Application No. 21741/93
by 1. Maximiliaan Maarten Pieter HAALEBOS and 79 others, and
2. De Nederlandse Vereniging voor Thoraxchirurgie;
against the Netherlands

The European Commission of Human Rights (Second Chamber) sitting in private on 11 January 1995, the following members being present:

Mr. H. DANELIUS, President
Mrs. G.H. THUNE
MM. G. JÖRUNDSSON
S. TRECHSEL
J.-C. SOYER
H.G. SCHERMERS
F. MARTINEZ
L. LOUCAIDES
J.-C. GEUS
M.A. NOWICKI
I. CABRAL BARRETO
J. MUCHA
D. SVÁBY

Mr. K. ROGGE, Secretary to the Chamber

Having regard to Article 25 of the Convention for the Protection of Human Rights and Fundamental Freedoms;

Having regard to the application introduced on 8 November 1992 by Maximiliaan Maarten Pieter HAALEBOS and 79 others and De Nederlandse Vereniging voor Thoraxchirurgie against the Netherlands and registered on 26 April 1993 under file No. 21741/93;

Having regard to the report provided for in Rule 47 of the Rules of Procedure of the Commission;

Having deliberated;

Decides as follows:

THE FACTS

The applicants are 80 cardio-pulmonary surgeons working in the Netherlands, and the Dutch Society for Thoracic Surgery (Nederlandse Vereniging voor Thoraxchirurgie, hereinafter referred to as the "NVT"), of which all of the above 80 cardio-pulmonary surgeons are members. The particulars of the 80 surgeons are appended to this report.

In the proceedings before the Commission, 68 of the 80 cardio-pulmonary surgeons are represented by Mr. E.C.M. Wagemakers, a lawyer practising in Breda, the Netherlands. The NVT acts on its own behalf and on behalf of its members. The NVT itself is represented by its President, Mr. M.M.P. Haalebos (applicant No. 28), and its Secretary Mr. T. Ebels (applicant No. 19), both of whom are represented by Mr. E.C.M. Wagemakers.

The facts of the case, as submitted by the applicants, may be summarised as follows.

1. General background

In 1982, the Health Care Tariffs Act (Wet Tarieven Gezondheids-zorg, hereinafter referred to as "WTG") came into force. This act aims

at a balanced system of determining health care tariffs in order to control the escalation of health care costs.

Pursuant to the WTG, health care tariffs are determined through negotiations between, on the one hand, the institutions paying for health care (public health services and medical insurance companies), and, on the other hand, medical institutions (such as, inter alia, hospitals, convalescent hospitals, maternity clinics) and persons providing health care (such as, inter alia, physicians, dentists, pharmacists, specialists).

In order to avoid having to negotiate with an ever increasing number of parties, the public authorities have fixed criteria for selection based on the idea of representativeness of organisations. Participation in tariff negotiations is limited to the organisations that have been recognised as representative organisations by the Ministry of Welfare, Public Health and Cultural Affairs (Ministerie van Welzijn, Volksgezondheid en Cultuur) pursuant to Section 3 of the WTG.

Each representative organisation represents a category of institutions or people within the health care system. In the Decree on Representative Organisations (Besluit houdende representatieve organisaties, hereinafter referred to as the "1984 Decree"), the categories of institutions and persons within the health care system are listed. Only for the categories listed in this Decree, representative organisations can be recognised. The Decree became effective in 1984, and has been amended in later years to include new categories of institutions or persons. The Decree prescribes, inter alia, that all medical specialists form one category; cardio-pulmonary surgeons are not mentioned as a separate category.

The criteria that must be met in order for an organisation to be recognised as representative are laid down in a public notice of the Deputy Minister of Welfare, Public Health and Cultural Affairs. Two of those criteria concern the number of members. The representative character of an organisation in this respect is assessed at two levels.

In the first place, the organisation must have a significant number of members in the group which it wishes to represent; secondly, the number of members should not be disproportionately small compared with the number of members of other organisations within the same category.

The recognised representative organisations, insofar as relevant for the present case, are the "Vereniging van Nederlandse Ziekenfondsen" (VNZ) - representing the public health services -, the "Kontaktorgaan Landelijke Organisaties van Ziektekostenverzekeraars" (KLOZ) - representing the private medical insurance companies -, the "Kontaktcommissie Publiekrechtelijke Ziektekostenregelingen voor Ambtenaren" (KPZ) - representing the medical insurance companies for civil servants -, the "Nationale Ziekenhuisraad" (NZR) - representing the medical institutions -, and the "Landelijke Specialisten Vereniging" (the National Association of Specialists, hereinafter referred to as "LSV") - representing the specialists.

Pursuant to the 1984 Decree, all specialists form one category. Within the category of specialists there are 26 sub-categories of which the cardio-pulmonary surgeons is one. As far as tariff negotiations are concerned, all specialists working in the Netherlands are represented by the LSV. The LSV has approximately 7,450 members. A number of cardio-pulmonary surgeons used to be members of the LSV.

The NVT used to be a separate organisation within the framework of the LSV. The NVT is a legal person under Dutch law, founded in 1948. According to Section 4 of its articles of association (statuten), the aims of the NVT are to advance the development of cardio-pulmonary surgery and to promote the interests of its members. Allegedly, all

cardio-pulmonary surgeons working in the Netherlands are NVT members.

Within the LSV, each of the 26 sub-categories of specialists has one vote. In addition to the 26 sub-categories of specialists, there are 20 geographical regions, each one having one vote.

The results of the tariff negotiations are sent to the Central Body for Health Care Tariffs (Centraal Orgaan Tarieven Gezondheidszorg, hereinafter referred to as the "COTG"), which approves or adjusts the tariffs. The COTG is a public body, whose independent members are appointed by the Government.

The COTG may be provided with guidelines on the level, the structure and the way of calculation of tariffs, which it should take into account in the exercise of its functions. These guidelines are determined by the Minister of Welfare, Public Health and Cultural Affairs, on the basis of advice by the COTG and in agreement with the Minister of Economic Affairs (Minister van Economische Zaken), the Minister of Social Affairs (Minister van Sociale Zaken) and other Ministers involved.

Pursuant to Section 2 of the WTG, it is a criminal offence to charge a tariff that has not been approved or determined by the COTG.

Pursuant to Section 35 of the WTG, an appeal can be lodged with the Industrial Appeals Tribunal (College van Beroep voor het Bedrijfsleven) against the COTG's decision to approve or adjust tariffs by, inter alia, individual specialists.

On 11 December 1989, the so-called "Five Parties Agreement" was concluded between the VNZ, the KLOZ, the KPZ, the NZR and the LSV. These five representative organisations stated in this agreement that the tariffs for operations performed by a number of sub-categories of specialists were either too low or too high, and did not reflect the required balance between the costs of the operation and the remuneration. The agreement aimed at correcting this unbalance. According to the agreement, the tariffs for operations performed by six sub-categories of specialists had to be lowered, the tariffs of six others raised, and the remaining fourteen could remain unchanged. The tariffs for operations performed by cardio-pulmonary surgeons had to be lowered by 30%, in three steps of 10% each.

On 26 January 1990, following the Five Parties Agreement, the NVT broke off all relations with the LSV. The NVT informed the LSV that it no longer wanted to be represented by the LSV in tariff negotiations. The applicants allege that, at present, not a single cardio-pulmonary surgeon working in the Netherlands is a LSV member.

2. The request of the NVT to be recognised as a representative organisation

On 22 March 1990, the NVT requested the Deputy Minister of Welfare, Public Health and Cultural Affairs to be recognised as the representative organisation within the meaning of Section 3 of the WTG for cardio-pulmonary surgeons. On 18 December 1990, the Deputy Minister rejected the request.

On 10 January 1991, the NVT lodged an objection (bezwaarschrift) against this decision. The Deputy Minister rejected the objection on 6 November 1991. He held that cardio-pulmonary surgeons were not mentioned in the 1984 Decree as a separate category and that, consequently, the NVT could not be recognised as the representative organisation for cardio-pulmonary surgeons. He further held that cardio-pulmonary surgeons formed only a sub-category of all specialists and that the NVT could not be considered as a representative organisation for all specialists, since, in absolute figures, it had few members and represented only a very small number of the specialists

working in the Netherlands.

Thereupon, the NVT lodged an appeal with the Judicial Division of the Council of State (Afdeling Rechtspraak van de Raad van State). In the proceedings before the Judicial Division, the NVT complained, inter alia, that the refusal to recognise it as a representative organisation was contrary to Articles 11 and 14 of the Convention.

On 10 November 1992, the Judicial Division of the Council of State rejected the appeal and upheld the Deputy Minister's reasoning and decision. It added that the NVT could request the Deputy Minister to amend the 1984 Decree to the effect that it would mention cardio-pulmonary surgeons as a separate category of persons within the health care system, and that the NVT could start civil proceedings in case the Deputy Minister would refuse that request.

As regards the alleged violation of Articles 11 and 14 of the Convention, the Judicial Division of the Council of State held:

<Translation>

> "Article 11 para. 1 of the Convention safeguards, inter alia, everyone's right to form with others trade unions and to join trade unions for the protection of his interests.
>
> From this provision can also be deduced the obligation of the State to sufficiently enable trade unions to promote the interests of their members.
>
> It follows from the aforementioned judgment [Eur. Court H.R., National Union of Belgian Police judgment of 27 October 1975] that Article 11 para. 1 of the Convention leaves each State free to choose the means to this end and that a policy which is aimed at limiting the number of organisations to be consulted is, in itself, not incompatible with this provision.
>
> The defendant [the Deputy Minister of Welfare, Public Health and Cultural Affairs] has submitted that his policy aims at limiting the number of representative organisations, inter alia, in order to make the tariff negotiations progress in an orderly fashion.
>
> Now that the specialists represented by the appellant [NVT] can join the LSV, and the appellant itself has opportunities to promote the interests of its members, for instance through contacts with the LSV, there is no reason to find this policy incompatible with Article 11 para. 1 of the Convention. ....
>
> The complaint that the decision is contrary to Article 11 read in conjunction with Article 14 of the Convention fails as well. ....
>
> This aforementioned complaint is founded on the argument that the defendant wrongly distinguishes between the LSV and the NVT.... This argument, however, cannot be accepted as correct, since the LSV cannot be put on a par with the NVT. The LSV is to be considered as an umbrella organisation, including the practitioners of all sorts of medical specialisations, whilst the appellant only represents a group of so-called super specialists. The Division finds furthermore that, even assuming [the organisations can be regarded as similar], there is an objective and reasonable justification for the difference in treatment."

3. The proceedings concerning the tariffs

On 5 December 1990, the LSV, the KLOZ, the KPZ and the VNZ requested the COTG to approve the tariffs on which they had reached agreement on 11 December 1989.

In two decisions of 17 December 1990, the COTG approved the new tariffs for, inter alia, operations performed by cardio-pulmonary surgeons. The new tariffs, which were applicable to services rendered to both persons falling within the scope of the national health care insurance and persons falling outside the scope of the national health care insurance (ziekenfondsverzekerden en niet-ziekenfondsverzekerden), were 10% lower than the tariffs previously charged.

After the introduction of the present application to the Commission, the tariffs for operations performed by cardio-pulmonary surgeons, in accordance with the Five Parties Agreement, were lowered by a further 20%.

On 7 January 1991, twelve cardio-pulmonary surgeons, among whom Mr. M.M.P. Haalebos (applicant No. 28), lodged an appeal against the COTG's decisions with the Industrial Appeals Tribunal (College van Beroep voor het Bedrijfsleven). They were all represented by the same lawyer. Their opponents in these proceedings were, inter alia, the COTG and the LSV.

In their written memorial to the Industrial Appeals Tribunal and at a hearing before the Tribunal, the twelve cardio-pulmonary surgeons argued that the decisions of the COTG to lower the tariffs were based on incorrect data concerning, inter alia, workload, turnover and operating expenses. They complained that the COTG, instead of asking them for additional and correct data, had based its decisions on the data provided by the LSV. They invoked Articles 6 and 11 of the Convention.

On 19 May 1992, the Industrial Appeals Tribunal rejected the appeal. It held that, as the LSV was the representative organisation for cardio-pulmonary surgeons and as the tariffs were based on the proposals and data of this organisation, the COTG had, in principle, no legal obligation to consult the NVT for additional data, and that in the present case there were no particular reasons on the basis of which the COTG should nevertheless have consulted the cardio-pulmonary surgeons.

In its decision the Industrial Appeals Tribunal had regard to the fact that the cardio-pulmonary surgeons had not produced data on turnover and workload demonstrating that the data on which the COTG had based its decisions were in fact incorrect. The Industrial Appeals Tribunal further noted that the cardio-pulmonary surgeons could have objected to the proposals within the LSV, but that they had not availed themselves of that opportunity.

COMPLAINTS

1. The applicants complain that the refusal to recognise the NVT as the representative organisation for cardio-pulmonary surgeons and the way the tariffs at issue were determined are contrary to Article 11 of the Convention. They allege that, by way of the WTG, the Government has given the LSV a monopoly position and that this makes it impossible for cardio-pulmonary surgeons to establish an organisation of their own which can effectively secure their specific interests in negotiations concerning the tariffs for cardio-pulmonary surgeons.

2. The applicants further complain under Article 6 para. 1 of the Convention that in the proceedings in which the tariffs were determined they did not receive a fair trial. They submit that, although they had demonstrated irrefutably that the COTG had based its tariff decisions on wrong data, the Industrial Appeals Tribunal rejected this argument holding that these data had been submitted by the LSV, the representative organisation for all specialists, including cardio-pulmonary surgeons.

THE LAW

1.a. The Commission observes that out of the 80 persons enumerated in the annex, only 68 have issued powers of attorney in favour of Mr. Wagemakers. The remaining 12 persons have not themselves presented any application nor have they issued powers of attorney to any other person to represent them before the Commission. However, the NVT has stated that it acts also on their behalf.

The question therefore arises whether the 12 persons can be considered to have lodged an application with the Commission.

However, the Commission does not find it necessary to determine this question in the circumstances of the present case, since it can examine the identical complaints lodged by the 68 applicants and since it considers these complaints to be inadmissible for the reasons set out hereafter.

b. The Commission further notes that not all the applicants were party to all the domestic proceedings concerned. The further question therefore arises whether those applicants who were not a party to certain domestic proceedings may still complain to the Commission of the outcome of these proceedings or the manner in which these proceedings were conducted. The Commission also does not find it necessary to determine this issue, since for the reasons set out below, it finds the various complaints to be inadmissible.

2. The first complaint is that the refusal to recognise the NVT as the representative organisation for cardio-pulmonary surgeons and the way the tariffs at issue were determined are contrary to Article 11 (Art. 11) of the Convention.

Article 11 (Art. 11) of the Convention reads as follows:

> "1. Everyone has the right to freedom of peaceful assembly and to freedom of association with others, including the right to form and to join trade unions for the protection of his interests.
>
> 2. No restrictions shall be placed on the exercise of these rights other than such as are prescribed by law and are necessary in a democratic society in the interests of national security or public safety, for the prevention of disorder or crime, for the protection of health or morals or for the protection of the rights and freedoms of others...."

The Commission recalls that, while Article 11 para. 1 (Art. 11-1) of the Convention presents trade union freedom as one form or a special aspect of freedom of association, the Article does not guarantee any particular treatment of trade unions, or their members, by the State, such as the right to be consulted by the State or the right that the State should conclude any given collective agreement with them (cf. Eur. Court H.R., National Union of Belgian Police judgment of 27 October 1975, Series A. No. 19, p. 17, para. 38; and Swedish Engine Drivers' Union judgment of 6 February 1976, Series A No. 20, p. 15, para. 39).

The Commission further recalls that the Convention safeguards freedom to protect the occupational interests of trade union members by trade union action, the conduct and development of which the Contracting State must both permit and make possible. It follows that the members of a trade union have a right, in order to protect their interests, that the trade union should be heard. Article 11 para. 1 (Art. 11-1) certainly leaves each State a free choice of the means to be used towards this end. While consultation and the concluding of collective agreements are two of these means, there are others. What the Convention requires is that under national law trade unions should

be enabled, in conditions not at variance with Article 11 (Art. 11), to strive for the protection of their members' interests (Eur. Court H.R., National Union of Belgian Police judgment, loc. cit., p. 18, para. 39; and Swedish Engine Drivers' Union judgment, loc. cit., p. 15, para. 39).

The Commission notes that the cardio-pulmonary surgeons are not forced to join the LSV or any other specific professional association. They are free to create their own professional association, which they did in fact in 1948 when the NVT was established, or not to associate themselves.

The NVT can engage in various activities to promote the interests of its members.

The Commission further notes that the Dutch legislature has created the system laid down in the WTG in order to control the escalation of health care costs. In order to keep the tariff negotiations as simple as possible, the legislature has chosen to limit the number of representative organisations to be consulted. The Commission finds that this system, in itself, cannot be regarded as incompatible with Article 11 (Art. 11) of the Convention (cf. Eur. Court H.R., National Union of Belgian Police case, loc. cit., p. 18, para. 41).

The Commission has also had regard to the fact that it is open to the NVT to seek to achieve a change in the 1984 Decree to the effect that cardio-pulmonary surgeons are included in the Decree as a separate category, which would subsequently create the possibility for the NVT to seek the status of a representative organisation for the purposes of the WTG. It does not appear, however, that the NVT has in fact tried to obtain a change in the 1984 Decree.

The Commission therefore finds that the fact that the NVT is not recognised as the representative organisation for cardio-pulmonary surgeons pursuant to Section 3 of the WTG - the consequence of which is that it cannot participate in tariff negotiations - does not in itself constitute a restriction on the exercise of the rights guaranteed by Article 11 para. 1 (Art. 11-1) of the Convention.

It follows that this part of the application is manifestly ill-founded within the meaning of Article 27 para. 2 (Art. 27-2) of the Convention.

3. The applicants further complain under Article 6 para. 1 (Art. 6-1) of the Convention that in the proceedings in which the tariffs for cardio-pulmonary interventions were determined they did not receive a fair trial, in that this determination was based on incorrect data.

Article 6 para. 1 (Art. 6-1) of the Convention, insofar as relevant, reads:

> "In the determination of his civil rights..., everyone is entitled to a fair...hearing...by an independent and impartial tribunal established by law...."

The Commission notes that the proceedings before the Industrial Appeals Tribunal were decisive for the fees cardio-pulmonary surgeons could charge. The Commission is of the opinion that these proceedings fall within the scope of Article 6 para. 1 (Art. 6-1) of the Convention, as they determined the cardio-pulmonary surgeons' civil rights and obligations within the meaning of this provision (cf. No. 11097/84, Dec. 18.7.86, D.R. 48 p. 204).

The applicants allege that the proceedings before the Industrial Appeals Tribunal were unfair, as the Tribunal based its decision on

incorrect data provided by the LSV to the COTG, the LSV being considered as the representative organisation for specialists, including cardio-pulmonary surgeons.

The Commission recalls that, in accordance with Article 19 (Art. 19) of the Convention, its only task is to ensure the observance of the obligations undertaken by the Parties to the Convention. In particular, it is not competent to deal with complaints that errors of law or fact have been committed by domestic courts, except where it considers that such errors might have involved a possible violation of any of the rights and freedoms set out in the Convention (cf. No. 17722/91, Dec. 8.4.91, D.R. 69 p. 345).

The Commission notes that the applicants, insofar as they were a party to those proceedings, could, and in fact did, challenge the correctness of the data on which the LSV's tariff proposals and the subsequent decisions of the COTG were based. The Commission however further notes that, according to the Industrial Appeals Tribunal, the allegation of incorrectness of the data remained unsubstantiated as the applicants failed to produce data on turnover and workload demonstrating that the data provided by the LSV, on which the COTG had based its decisions, were incorrect.

In these circumstances, the Commission cannot find that the decision by the Industrial Appeals Tribunal as regards the reliability of the data before it is unreasonable or arbitrary.

Furthermore, noting that in the proceedings before the Industrial Appeals Tribunal the applicants, who were represented by a lawyer, have been provided with ample opportunity to state their case both orally and in writing, to submit relevant information and to challenge the arguments of the adversary parties in those proceedings, the Commission finds no indication that the proceedings were not in conformity with the requirements of Article 6 para. 1 (Art. 6-1) of the Convention.

It follows that this part of the application is also manifestly ill-founded within the meaning of Article 27 para. 2 (Art. 27-2) of the Convention.

For these reasons, the Commission, by a majority,

DECLARES INADMISSIBLE THE COMPLAINT BROUGHT UNDER ARTICLE 11 OF THE CONVENTION;

and, unanimously,

DECLARES INADMISSIBLE THE REMAINDER OF THE APPLICATION.

Secretary to the Second Chamber — President of the Second Chamber

(K. ROGGE) — (H. DANELIUS)

ANNEX

list of applicants

Name:	Place of residence	Represented by Mr.Wagemakers
1. Z. Aytug	Leiderdorp	Yes
2. R. Bakhuizen	Wapenveld	Yes
3. P.F.A. Bakker-de Wekker	Hilversum	Yes
4. J.H. Bavinck	Eindhoven	No
5. J.A. Bekkers	Rotterdam	Yes
6. E. Berreklouw	Son	Yes
7. K. Bloemendaal	Abcoude	Yes
8. W.J. de Boer	Haren	Yes
9. A.J.J.C. Bogers	Oegstgeest	Yes

10. P.W. Boonstra	Groningen	Yes
11. E. Bos	Rotterdam	Yes
12. W.G.B. Brands	Ulvenhout	Yes
13. J.J. Bredée	Utrecht	Yes
14. E. Broekhuis	Amsterdam	Yes
15. M.H.J. Brouwer	Haren	Yes
16. A. Brutel de la Rivière	Nieuwegein	Yes
17. J.J.A.M. Defauw	Wijk bij Duurstede	Yes
18. R.A.E. Dion	Bierbeek (Belgium)	Yes
19. T. Ebels	Haren	Yes
20. P.S. Eggens	Breda	Yes
21. A. Eijgelaar	Groningen	No
22. L. Eijsman	Bussum	Yes
23. Th.W.O. Elenbaas	Maastricht	Yes
24. R. de Geest	Aalst (Belgium)	No
25. Th.R. van Geldorp	Breda	Yes
26. G.G. Geskes	Maastricht	Yes
27. J.G. Grandjean	Haren	Yes
28. M.M.P. Haalebos	Hattem	Yes
29. R. Hamerlijnck	Zeist	Yes
30. R. Hardjowijono	Huizen	Yes
31. K. ten Have	Amsterdam	Yes
32. M.G. Hazekamp	Noordwijkerhout	Yes
33. L.A. van Herwerden	Rotterdam	Yes
34. P.A.E. Hiddema	Rotterdam	Yes
35. J.F. Hitchcock	Utrecht	No
36. M.A.J.M. Huybregts	Diemen	Yes
37. H.A. Huysmans	Oegstgeest	Yes
38. G.J. van Ingen	Passau (Germany)	Yes
39. B. Jambroes	Utrecht	Yes
40. E.W.L. Jansen	Zeist	Yes
41. A.P. de Jong	Haarlem	Yes
42. G.L. Kaan	Malden	Yes
43. J.J. Kloek	Koog aan de Zaan	Yes
44. P.J.J.M.L. Knaepen	Bilthoven	Yes
45. G.J. Kootstra	Breda	Yes
46. L.K. Lacquet	Ubbergen	Yes
47. J.R. Lahpor	Utrecht	Yes
48. F. Leicher	Hattem	Yes
49. A.P.W.M. Maat	Rotterdam	Yes
50. Y.A.S. Mashhour	Lieshout	Yes

Name:	Place of residence	Represented by Mr.Wagemakers
51. J.J. van der Meer	Ulvenhout	Yes
52. J. van der Meulen	Nijmegen	Yes
53. J.J. Meuzelaar	Noordwijk	Yes
54. H.G. Meyne	Amsterdam	No
55. B. Mochtar	Rotterdam	Yes
56. B.A.J.M. de Mol	Vianen	Yes
57. W.J. Morshuis	Culemborg	Yes
58. A.C. Moulijn	Beuningen	Yes
59. A. Nijveld	Hoofddorp	Yes
60. L. Noyez	Nijmegen	Yes
61. O.C.K.M. Penn	Maastricht	Yes
62. K.B. Prenger	Meerssen	Yes
63. J.M. Quagebeur	not stated	No
64. G.L. van Rijk-Zwikker	Heemstede	Yes
65. A. Rocher	Brugge (Belgium)	No
66. J.P.A.M. Schönberger	Someren	Yes
67. T.H. Sie	Hattem	Yes
68. S.H. Skotnicki	Nijmegen	Yes
69. A.H.M. van Straten	Oirschot	Yes
70. H.A. van Swieten	Zeist	Yes
71. F.E.E. Vermeulen	Nieuwegein	Yes
72. M.I.M. Versteegh	Sassenheim	Yes

73. J.G. Vincent	Nijmegen	No
74. M.M. Vrakking	Leiden	Yes
75. H.J. van der Wal	Milsbeek	No
76. T.W. Waterbolk	Haren	Yes
77. H.P. Wijnen	Le Muids (Switzerland)	No
78. C.R.H. Wildevuur	Groningen	No
79. S.R. Woolley	Utrecht	Yes
80. B.S. Zienkowicz	Amsterdam	No